

Clinical Findings

The clinical spectrum includes cryptic infection, localized cutaneous disease (CL), and disseminated infection (DCL, MCL, and VL).

Cutaneous Leishmaniasis

Cutaneous leishmaniasis (CL) constitutes 50% to 75% of all incident cases and is the mildest form of the disease. It can be caused by any *Leishmania* species. Most cases heal spontaneously within 1 year and are classified as acute CL. Disease lasting more than 1 year is termed chronic CL (CCL).

Old World Cutaneous Leishmaniasis

Old World cutaneous leishmaniasis (OWCL), also known as Aleppo boil, Baghdad boil, Oriental sore, *leishmaniasis tropica*, Biskra button, Delhi boil, Bouton d'orient, Lahore sore, rose of Jericho, Kandahar sore, or "the little sister," is caused mainly by *L. major*, *L. tropica*, and *L. aethiopica*, and to a lesser extent by *L. infantum*. Two major types are recognized.

Moist or Rural Type

The moist or rural type is classically caused by *L. major* and is characterized by: - Multiple lesions - Short incubation period (1 week to 3 months) - Rapid and mild clinical course - Good response to therapy

Rodents are the primary reservoirs. After bites from infected sand flies, one or more (up to 100) insect bite-like lesions appear, typically on exposed areas such as the face, arms, and legs. Most resolve within 1–2 weeks; only a few progress to typical CL. These evolving lesions are painless and usually heal within 6 months, leaving atrophic scars.

Dry or Urban Type

The dry or urban type is classically caused by *L. tropica* and is characterized by: - Longer incubation period and disease duration (approximately twice as long as the moist type) - Poorer response to therapy - Anthroponotic transmission (human-to-human via sand flies) - Potential for persistence, progression, recurrence, and visceralization

A single "dry" ulcer, typically on the face, is characteristic.

These two types may coexist in the same patient. *L. aethiopica* may cause a similar cutaneous disease but carries a higher risk of progressing to diffuse cutaneous leishmaniasis (DCL). *L. infantum*, the primary cause of Mediterranean visceral leishmaniasis in children, can cause a self-limited cutaneous disease in adults.

Species identification cannot be made based on clinical features alone and requires biochemical or molecular diagnostic methods.

Morphologic Spectrum of OWCL

Lesions typically begin as erythematous papules that enlarge over several weeks into nodules or plaques. These often ulcerate and become crusted. The classic "volcanic" noduloulcerative lesion is painless and features: - A crateriform ulcer - A rolled border - A necrotic base - An adherent crust

Other clinical presentations include: - Iceberg nodules - Eczematoid - Psoriasiform - Erysipeloid - Zosteriform - Paronychial - Chancriform - Annular - Palmoplantar - Verrucous - Keloidal lesions

Satellitosis, regional lymphadenopathy, localized lymphadenitis, and sporotrichoid lymphatic spread may also occur.